

Started on	Sunday, 12 February 2023, 8:17 PM
State	Finished
Completed on	Sunday, 12 February 2023, 8:17 PM
Time taken	13 secs
Marks	0.00/39.00
Grade	0.00 out of 100.00

Question **1**

Not answered

Marked out of 1.00

Catastrophic reaction is best described as a/an

Select one:

- ☐ Hysterical reaction to stressors seen in anxiety states
- ☐ Response to a cognitive challenge seen in patients with dementia
- ☐ Acute stress reaction
- ☐ Response to traumatic recall seen in patients with PTSD
- ☐ Response to threat seen in somatisation

Your answer is incorrect.

Explanation:

Catastrophic reaction is a response occasionally seen in patients with dementia who are asked to perform tasks beyond their, now impaired, performance level. There is sudden agitation, anger, and occasionally violence.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 104.

The correct answer is: Response to a cognitive challenge seen in patients with dementia

Question 2

Not answered

Marked out of 1.00

Mr X is a 27-year-old man admitted to a psychiatric unit for treatment of his psychotic illness. His presentation fluctuates. At time, he is restless, agitated and excited. At other times, he is mute, immobile and demonstrates waxy flexibility. Which of the following medications is most likely to help him acutely?

Select one:

- ☐ Lorazepam
- ☐ Risperidone
- ☐ Benztropine
- ☐ Propanolol
- ☐ Procyclidine

Your answer is incorrect.

Explanation:

This man has catatonia, alternating between excited and stuporous states. In most catatonic patients, inpatient care will be required. The use of lorazepam (1–2 mg, sublingually, intramuscularly, or intravenously) is recommended, in addition to treatment of the underlying syndrome. ECT is also effective in the treatment of catatonia.

Subtypes of catatonia:

1. **Stuporous/retarded:** associated with signs reflecting a paucity of movement, including immobility, mutism, staring, rigidity, withdrawal, and refusal to eat, along with more bizarre features such as catalepsy, posturing, grimacing, negativism, waxy flexibility, echolalia or echopraxia, stereotypy, verbigeration (repetitive speech, like a scratched record), automatic obedience (exaggerated compliance with commands), and ambitendence (indecisive, hesitant movements)
2. **Excited/delirious:** characterised by severe psychomotor agitation, potentially leading to life-threatening complications such as hyperthermia, altered consciousness, and autonomic dysfunction ("malignant" or "lethal" catatonia)

Ref: Rasmussen SA et al. Catatonia: our current understanding of its diagnosis, treatment and pathophysiology. World journal of psychiatry. 2016 Dec 22;6(4):391.

Taylor DM, Gaughran F, Pillinger T. The Maudsley Practice Guidelines for Physical Health Conditions in Psychiatry. 2021. pp. 441-448.

The correct answer is: Lorazepam

Question **3**

Not answered

Marked out of 1.00

What is the proportion of individuals with epilepsy that commit suicide?

Select one:

- ☐ 30%
- ☐ 50%
- ☐ 20%
- ☐ 10%
- ☐ 40%

Your answer is incorrect.

This elevated risk is seen even in the absence of diagnosable depression.

The correct answer is: 10%

Question 4

Not answered

Marked out of 1.00

The estimate of lifetime suicide prevalence in schizophrenia patients observed either from first admission or illness onset is

Select one:

- ☐ 6%
- ☐ 10%
- ☐ 15%
- ☐ 2%
- ☐ 0.8%

Your answer is incorrect.

The suicide rate in schizophrenia was thought to be 10% for a long time But a recent meta-analysis which included studies that were continued for at least 2 years with minimum 90% follow-up rates concluded differently (Palmer, Arch Gen Psych 2005). The estimate of lifetime suicide prevalence in those observed from first admission or illness onset was 5.6% (95% confidence interval, 3.7%-8.5%).

The correct answer is: 6%



Question 5

Not answered

Marked out of 1.00

Rates of completed suicide are clearly raised in all of the following except

Select one:

- ☐ Alcohol dependence
- ☐ Depression
- ☐ Mania
- ☐ Schizophrenia
- ☐ Dementia

Your answer is incorrect.

Explanation:

The association between dementia and suicidal behaviour remains poorly understood. Some studies have found no association or even a lower risk of suicide in those with dementia, whereas other studies have found an increased risk of suicide, particularly in patients recently diagnosed with dementia. Patients with a recent diagnosis for MCI or dementia may have preserved insight into what such a diagnosis entails, anticipating progressive cognitive and functional decline, fearing loss of autonomy, and worrying that they may become a burden to significant others. Perceived burdensomeness to others is a crucial interpersonal construct that may influence desire to die by suicide. On the other hand, patients with prior diagnoses of MCI or dementia may be less likely to prepare and carry out a suicide plan and may also use less lethal means (either by choice or due to lack of access).

Ref: Günak MM et al. Risk of suicide attempt in patients with recent diagnosis of mild cognitive impairment or dementia. JAMA psychiatry. 2021 Jun 1;78(6):659-66.

The correct answer is: Dementia

Question 6

Not answered

Marked out of 1.00

The most common psychiatric diagnosis associated with suicide is

Select one:

- ☐ Alcohol dependence
- ☐ Mania
- ☐ Schizophrenia
- ☐ Dementia
- ☐ Depression

Your answer is incorrect.

The most common psychiatric diagnoses in suicide are major depression (30-31%) and alcohol dependence (17-24%).

The correct answer is: Depression



Question 7

Not answered

Marked out of 1.00

Most countries have a higher male suicide rates than female suicide rates. This is not the case in

Select one:

- ☐ Japan
- ☐ China
- ☐ UK
- ☐ Russia
- ☐ Italy

Your answer is incorrect.

The male: female ratio is 3.5:1 for completed suicides globally. An exception is China where females have higher or comparable rates to males. Number of suicides in China is 30% greater than that of whole Europe - due to the size of the population.

The correct answer is: China



Question 8

Not answered

Marked out of 1.00

According to the survey of psychiatric morbidities among adults living in private households, the lifetime prevalence of deliberate self harm in patients with a possible psychotic disorder is

Select one:

- ☐ 0.25
- ☐ 0.15
- ☐ 0.35
- ☐ 0.05
- ☐ 0.45

Your answer is incorrect.

The lifetime prevalence of DSH in patients with a possible psychotic disorder is 25% (ONS statistics, 2007).

The correct answer is: 0.25



Question 9

Not answered

Marked out of 1.00

Which of the following statements regarding suicide in bipolar disorder is false?

Select one:

- ☐ Suicide rates in men hospitalised for bipolar disorder are 15 times higher than the general population
- ☐ Nearly 25% of people with bipolar II disorder attempt suicide
- ☐ Most completed suicides occur in the manic phase of bipolar disorder
- ☐ 5-8% of people with bipolar disorder die by suicide
- ☐ Suicide attempts are more prevalent in bipolar disorder than in unipolar depression

Your answer is incorrect.

Explanation:

The all-cause standardized mortality ratio (SMR) is elevated in bipolar disorder relative to the general population. The risk of suicide is greatly elevated during depressive episodes in bipolar disorder. Most suicide attempts and most completed suicides occur in the depressed phase of the illness. Approximately 17% of people with bipolar I disorder and 24% with bipolar II disorder attempt suicide during the course of their illness; that is higher than in unipolar depression. Approximately 8% of men and 5% of women with bipolar disorder die by suicide. The SMR for suicide in bipolar disorder is 15 for men and 22.4 for women in those who have been hospitalised for bipolar disorder.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the assessment and management of bipolar disorder in adults, children and young people in primary and secondary care. Updated edition 2020.

The correct answer is: Most completed suicides occur in the manic phase of bipolar disorder

Question **10**

Not answered

Marked out of 1.00

What is the risk of suicide in a patient with schizophrenia (expressed in percentage)?

Select one:

- ☐ 1
- ☐ 10
- ☐ 6
- ☐ 3
- ☐ 20

Your answer is incorrect.

Suicide claims the lives of about 6% of schizophrenic patients. They are more likely to injure themselves than others. Between 2% and 12% of all individuals dying by suicide have schizophrenia. (Heila et al 1997).

The correct answer is: 6



Question 11

Not answered

Marked out of 1.00

The prevalence of suicidal ideations in the past year among school-going adolescent in Britain is

Select one:

- ☐ 25%
- ☐ 3%
- ☐ 15%
- ☐ 5%
- ☐ 10%

Your answer is incorrect.

Suicidal ideation (without deliberate self harm) in the past year was reported by 15.0% of an adolescent cohort in UK (school pupils - self report). This was more common in females (22%) than males (8.5%) (Odds ratio 3.1).

The correct answer is: 15%



Question 12

Not answered

Marked out of 1.00

Which of the following diagnoses is associated with an increased risk of suicide?

Select one:

- ☐ Late paraphrenia
- ☐ Post schizophrenic depression
- ☐ Catatonic schizophrenia
- ☐ Paranoid schizophrenia
- ☐ Simple schizophrenia

Your answer is incorrect.

Some patients with schizophrenia develop depressive features within 12 months of an acute episode of psychosis. The depressive features develop in the presence of residual or active features of schizophrenia and are associated with an increased risk of suicide. It may be difficult to differentiate from negative symptoms or Parkinsonism and is associated with an increased risk of suicide.

The correct answer is: Post schizophrenic depression

Question 13

Not answered

Marked out of 1.00

What is the risk of dying by suicide in the next 2 years for a patient who has deliberately self-harmed today?

Select one:

- ☐ 1 in 1000
- ☐ 1 in 10
- ☐ 1 in 100
- ☐ 1 in 10 000
- ☐ 1 in 100 000

Your answer is incorrect.

Explanation:

Roughly 1% of patients will die by completed suicide in the 24 months following an initial act of self-harm, with the risk being highest in the weeks following the original act. This represents a mortality by suicide 50-100 times that of the general population.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 848.

The correct answer is: 1 in 100



Question **14**

Not answered

Marked out of 1.00

Risk factors for completed suicide include all except

Select one:

- ☐ Widowed
- ☐ Living alone
- ☐ Female sex
- ☐ Elderly
- ☐ Unemployed

Your answer is incorrect.

Explanation:

Sociodemographic risk factors for completed suicide include: male sex; elderly; single, divorced, or widowed; living alone, poor social support; unemployed or low socio-economic class.



Risk factors for suicide	
Demographic factors	Male; increasing age; low socioeconomic status; unmarried, separated, widowed; living alone; unemployed
Background history	Self harm (especially with high suicidal intent); repeated cutting in adolescence; childhood adversity (i.e. sexual/physical abuse, neglect, parental loss); family history of suicide; family history of mental illness; prior suicide attempt *Nonfatal suicidal behaviour is among the most robust predictors of future suicidal behaviour and suicide death
Clinical history	Mental illness diagnosis (e.g. depression, bipolar disorder, schizophrenia); personality disorder diagnosis; physical illness (especially chronic conditions and/or those associated with pain and functional impairment); recent contact with psychiatric services; recent discharge from psychiatric inpatient facility
Psychological and psychosocial factors	Hopelessness; impulsiveness; low self-esteem; life event; perfectionism; relationship instability; lack of social support
Current context	Suicidal ideation; suicide plan; availability of means; lethality of means; recent severe, stressful life events; job loss; financial problems; legal problems
Substance use	Intoxication; use of multiple substances; withdrawal from cocaine, amphetamines and other addictive drugs; extended use of sedatives, hypnotics, and anxiolytics
Adapted from Correia and Jackson 2020	

SPMM COURSE



Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 849.

Correia R, Jackson D. Risk to self: identifying and managing risk of suicide and self-harm. BJPsych Advances. 2020:1-11.

The correct answer is: Female sex

Question 15

Not answered

Marked out of 1.00

What percentage of people that die by suicide have a recognizable psychiatric disorder at the time of death?

Select one:

- ☐ 70%
- ☐ 10%
- ☐ 90%
- ☐ 50%
- ☐ 30%

Your answer is incorrect.

Explanation:

Mental illness is a key antecedent to suicide. In high-income countries (such as the UK), the proportion of people experiencing a mental illness at the time of their suicide is as high as 90%.

Ref: Correia R, Jackson D. Risk to self: identifying and managing risk of suicide and self-harm. BJPsych Advances. 2020;1-11.

Turecki G et al. Suicide and suicide risk. Nature reviews Disease primers. 2019 Oct 24;5(1):1-22.

The correct answer is: 90%



Question 16

Not answered

Marked out of 1.00

Catatonia refers to increased muscle tone that is

Select one:

- ☐ Present on active movements only
- ☐ Absent on passive movements only
- ☐ Present on both active and passive movements
- ☐ Reduced on both active or passive movement
- ☐ Present on passive movements only

Your answer is incorrect.

Psychomotor manifestations of catatonia analyzed by latent class techniques have been divided into 4 classes as follows: automatic, repetitive/echo, withdrawal, and agitated/resistive.

The correct answer is: Reduced on both active or passive movement



Question **17**

Not answered

Marked out of 1.00

Which of the following patients is at the highest risk of suicide?

Select one:

- ☐ 44 year old father of 2 children in a heterosexual partnership
- ☐ 35 year old widower, unemployed, social class V
- ☐ 60 year old man, married, social class V
- ☐ 30 year old woman, part-time employed, social class I
- ☐ 16 year old single, unmarried, social class I

Your answer is incorrect.

Middle aged men are at a higher risk compared to any other group described

The correct answer is: 60 year old man, married, social class V



Question 18

Not answered

Marked out of 1.00

Which of the following statements regarding the epidemiology of suicide is false?

Select one:

- ☐ Rates of completed suicide are higher in males than females
- ☐ Rates of suicide attempts are higher in females than males
- ☐ Across countries, the most common method of suicide remains the same
- ☐ Mental illness increases the risk of suicide
- ☐ Completed suicides are less common than attempted suicides

Your answer is incorrect.

Explanation:

Suicide is the 14th leading cause of death worldwide, responsible for 1.5% of global mortality. For every death by suicide, approximately 20 people make suicide attempts (rates vary by country, depending on the lethality of commonly used suicide methods). Poisons commonly ingested in high-income countries (e.g. analgesics and psychotropics medications) are considerably less toxic than pesticides, which are commonly used in suicide attempts in many low-income countries. Rates of suicide attempts are generally higher in females than in males, but in most countries rates of suicide are 2-3 times higher in males than females, probably due to male preference for higher lethality methods and reluctance of males to seek help. Mental illness is a key antecedent to suicide. In high-income countries (such as the UK), the proportion of people experiencing a mental illness at the time of their suicide is as high as 90%.

Ref: Correia R, Jackson D. Risk to self: identifying and managing risk of suicide and self-harm. BJPsych Advances. 2020;1-11.

Turecki G et al. Suicide and suicide risk. Nature reviews Disease primers. 2019 Oct 24;5(1):1-22.

The correct answer is: Across countries, the most common method of suicide remains the same

Question 19

Not answered

Marked out of 1.00

The most common risk factor for suicide attempts according to the Psychiatric Morbidity Survey conducted by the Office of National Statistics is

Select one:

- ☐ Social class 5
- ☐ South Asian ethnicity
- ☐ White British ethnicity
- ☐ African-Caribbean ethnicity
- ☐ Age more than 65

Your answer is incorrect.

Low socio-economic class (social class 5) is one the most common risk factor for suicide attempts according to Psychiatric morbidity survey

The correct answer is: Social class 5



Question 20

Not answered

Marked out of 1.00

Mr X is a 27 year old man who has a well-established diagnosis of schizophrenia. He was started on a new antipsychotic medication 2 weeks ago. Today he is brought to the A & E department after his community psychiatric nurse called for an ambulance. He has a temperature of 40°C and moderately severe muscular rigidity on examination. His pulse rate is raised, blood pressure is elevated, and he has a leucocytosis. He is unable to give a proper history as he appears confused and disoriented. Which of the following medications is most likely to be helpful in the pharmacological treatment of this man?

Select one:

- ☐ Valproate
- ☐ Lithium
- ☐ Propranolol
- ☐ Bromocriptine
- ☐ Haloperidol

Your answer is incorrect.

Explanation:

BAP guidelines (2020) state that neuroleptic malignant syndrome, if suspected, is a medical emergency. Offending neuroleptics must be stopped immediately and supportive treatment should be instituted (rehydration, electrolyte correction, external cooling if needed). Intensive care may be needed. The most widely recommended initial intervention is a high-potency benzodiazepine such as lorazepam (1-2mg IV). With more marked disorder (characterised by moderately severe rigidity, temperatures up to 40C, and pulse up to 120bpm), attempts to specifically release dopaminergic 'lock-down' are recommended, with either the dopamine agonist bromocriptine or the antimuscarinic agent amantadine. With severe and/or rapid progression, the peripheral muscle relaxant dantrolene may be tried. ECT is recommended in severe or refractory cases.

**Cardinal features:**

- **Exposure to a dopamine antagonist** within 72 hours prior to symptom development
- **Hyperthermia** (>38C orally on at least two occasions)
- **Generalised rigidity** ("lead pipe" in most severe form)

Other possible signs:

- **Neurological:** tremor, sialorrhea, akinesia, dystonia, trismus, dysarthria, dysphagia, rhabdomyolysis
- **Changes in mental status:** delirium, altered consciousness (ranging from stupor to coma), or catatonic stupor
- **Autonomic activation/instability:** tachycardia, diaphoresis, blood pressure elevation or fluctuation, urinary incontinence
- **Tachypnea/respiratory distress**
- **CK elevation** (at least four times above upper normal limit)
- **Other possible lab findings:** leukocytosis, metabolic acidosis, hypoxia, decreased serum iron concentrations, elevations in serum muscle enzymes and catecholamines

Neuroleptic Malignant Syndrome

SPMM COURSE



Ref: American Psychiatric Association. Diagnostic and statistical manual of mental disorders (DSM-5®). American Psychiatric Pub; 2013 May 22.

Barnes TR et al. Evidence-based guidelines for the pharmacological treatment of schizophrenia: Updated recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2020 Jan;34(1):3-78.

The correct answer is: Bromocriptine

Question **21**

Not answered

Marked out of 1.00

A 25-year-old man has recently returned from a business trip in the United States of America. He presents with aggressive behaviour. He recently developed seizures and has periods of decreased consciousness. His full blood count, lumbar puncture and liver functions are normal. Which of the following is the most likely diagnosis?

Select one:

- ☐ Huntington disease
- ☐ Complex partial seizures
- ☐ Hepatic encephalopathy
- ☐ Herpes simplex encephalitis
- ☐ Alcohol withdrawal

Your answer is incorrect.

In complex partial seizures, investigations are normal. Decreased periods of consciousness is common. Complex partial seizures begin as simple partial seizures and progress to impairment of consciousness. There may be also impairment of consciousness at the onset. Aggressive behaviour is also a possibility in patients with complex partial seizures.

The correct answer is: Complex partial seizures



Question **22**

Not answered

Marked out of 1.00

In which decade was ECT introduced into psychiatric treatment for the first time?

Select one:

- ☐ 1970s
- ☐ 1950s
- ☐ 1960s
- ☐ 1930s
- ☐ 1940s

Your answer is incorrect.

Explanation:

Convulsive therapies for major psychiatric illnesses predate the modern therapeutic era, with the use of camphor reported as early as the 16th century and the existence of several accounts of camphor convulsive therapies from the late 1700s to the mid-1800s. In 1934, Ladislav von Meduna induced seizures with injections of camphor-in-oil in the first catatonic psychotic patient who eventually made a full recovery. In 1938, Ugo Cerletti and Lucio Bini conducted the first electrical induction of a series of seizures in a delusional, incoherent patient and produced a successful treatment response.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 295.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 729.

The correct answer is: 1930s

Question 23

Not answered

Marked out of 1.00

Which of the following groups is at the lowest risk for developing acute dystonia?

Select one:

- ☐ Patients who are prescribed high potency antipsychotics
- ☐ Previously neuroleptic-naive patients now exposed to antipsychotics
- ☐ Young male patients
- ☐ Patients who have been recently started on antipsychotic therapy
- ☐ Young female patients

Your answer is incorrect.

Explanation:

Acute dystonia is an involuntary, intermittent or sustained muscular contraction causing slow, often twisting movements or fixed postures. It is commonly caused by antipsychotic medications (especially typical antipsychotics); other pharmacological triggers include antidopaminergic anti-emetics, serotonergic antidepressants (occasionally), and withdrawal of anticholinergic drugs used as treatment for dystonia. Known risk factors include male sex, younger age (especially children and young adults), recent alcohol use, recent cocaine use, prior dystonia, dehydration, hypocalcaemia, and use of high potency antipsychotics. Acute dystonia usually occurs within 1 week of commencing or rapidly increasing the dose of antipsychotic medication or of reducing the anticholinergic medication given to treat it.

Ref: Taylor DM, Gaughran F, Pillinger T. The Maudsley Practice Guidelines for Physical Health Conditions in Psychiatry. 2021. pp. 673-674.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 1016.

The correct answer is: Young female patients

Question **24**

Not answered

Marked out of 1.00

Considering the epidemiology of suicide, which of the following is true?

Select one:

- ☐ During economic recessions, suicide rates decrease
- ☐ Suicide-related statistics have low reliability
- ☐ All industrialised nations have comparable suicide rates
- ☐ During periods of war, suicide rates increase
- ☐ Suicide has been declining among young men in the UK

Your answer is incorrect.

Explanation:

The incidence of suicide varies more than 10-fold between countries. Several factors are believed to account for these differences in suicide rates, including variations in the accuracy of suicide statistics (inconsistent recording of suicides by coroners and cultural or religious acceptability of suicide can affect the reporting). Key influences in suicide trends include: periods of economic recession (associated with increases in suicide); changes in access to commonly used and high-lethality suicide methods; periods of war (generally associated with declines in suicide); and media reporting of celebrity suicides (may lead to transient increases in suicide rates). Suicide is currently the leading cause of death among young and middle-aged men in the UK (men have likely been more affected in recent years by economic adversity, alcoholism, and social isolation). In the UK, death rates among under 25s have increased in recent years.

Ref: Correia R, Jackson D. Risk to self: identifying and managing risk of suicide and self-harm. BJPsych Advances. 2020;1-11.

Turecki G et al. Suicide and suicide risk. Nature reviews Disease primers. 2019 Oct 24;5(1):1-22.

The correct answer is: Suicide-related statistics have low reliability

Question **25**

Not answered

Marked out of 1.00

Which one of the following diagnosis best applies to an 18-year-old girl who suddenly developed akinetic mutism, narrowed consciousness within few hours? She has no history of previous depression.

Select one:

- ☐ Dissociative stupor
- ☐ Depressive stupor
- ☐ Brief psychotic episode
- ☐ Acute stress reaction
- ☐ Manic stupor

Your answer is incorrect.

Dissociative stupor is diagnosed on the basis of a profound diminution or absence of voluntary movement and normal responsiveness to external stimuli such as light, noise, and touch, but examination and investigation reveal no evidence of a physical cause. In addition, there is positive evidence of psychogenic causation in the form of recent stressful events or problems according to ICD -10

The correct answer is: Dissociative stupor

Question **26**

Not answered

Marked out of 1.00

Delirium tremens is characterized by onset after

Select one:

- ☐ 24-48 hours of abstinence
- ☐ 7 days of abstinence
- ☐ 24 hours of abstinence
- ☐ 7 days of abstinence
- ☐ 48-72 hours of abstinence

Your answer is incorrect.

Delirium tremens is characterized by onset within 48-72 hours of abstinence, with disturbance worse at night. It is characterized by hallucinations of multiple modalities.

The correct answer is: 48-72 hours of abstinence



Question **27**

Not answered

Marked out of 1.00

Which one of the following statements is correct about mental capacity?

Select one:

- ☐ Capacity can only be assessed by a doctor
- ☐ Capacity refers to general decision-making ability
- ☐ Capacity may be absent for one task but present for another
- ☐ Capacity cannot be assessed if the patient is psychotic
- ☐ Capacity can only be assessed by a psychiatrist

Your answer is incorrect.

Explanation:

Capacity is a legal concept, meaning the ability to enter into valid contracts. Incapacity law and common law in the UK presume capacity in adults. Capacity is not an 'all-or-nothing' quality (i.e. one may have capacity for some decisions, but not others). Incapacity must be assessed in relation to the particular decision that is required of the patient. Incapacity cannot be presumed on the basis of any individual patient factors (e.g. diagnosis of dementia, intellectual disability, brain injury, mental disorder, psychotic disorder) and must be assessed specifically for each decision. All doctors should be familiar with the assessment of capacity.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 856-857, 936.

Murray BJ. Mental capacity: different models and their controversies. BJPsych Advances. 2017 Nov;23(6):366-74.

The correct answer is: Capacity may be absent for one task but present for another

Question **28**

Not answered

Marked out of 1.00

Paul has acute onset auditory and visual hallucinations despite clear consciousness. His heart rate and temperature are elevated, and he is quite irritable. His presentation is most likely to be caused by

Select one:

- ☐ Alcohol withdrawal
- ☐ Atropine poisoning
- ☐ Mushroom poisoning
- ☐ Amphetamine abuse
- ☐ Delirium

Your answer is incorrect.

Explanation:

Acute harmful effects of amphetamines include tachycardia, arrhythmias, hyperpyrexia, irritability, post-use depression, and a quasi-psychotic state with visual, auditory, and tactile hallucinations.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 622.

The correct answer is: Amphetamine abuse



Question **29**

Not answered

Marked out of 1.00

What percentage of individuals with dementia demonstrate inappropriate sexual behaviours?

Select one:

- ☐ 40-50%
- ☐ 2-5%
- ☐ 1-2%
- ☐ 7-25%
- ☐ 30-40%

Your answer is incorrect.

Explanation:

Inappropriate sexual behaviours are seen in 7-25% of individuals with dementia. This commonly includes inappropriate sexual talk, sexual acts, and implied sexual acts.

Ref: Srinivasan S et al. Sexuality and the Older Adult. Current Psychiatry Reports. 2019 Oct 1;21(10):97.

The correct answer is: 7-25%



Question **30**

Not answered

Marked out of 1.00

The number of people who attend their primary care physician (GP) in the week preceding their suicide is

Select one:

- ☐ 1 in 10
- ☐ 3 in 5
- ☐ 2 in 5
- ☐ 4 in 5
- ☐ 1 in 5

Your answer is incorrect.

Approximately one-quarter of individuals who complete suicide have been in contact with mental health services. In the week preceding suicide, 40% of patients make contact with their GPs.

The correct answer is: 2 in 5



Question **31**

Not answered

Marked out of 1.00

Which of the following is not a catatonic sign?

Select one:

- ☐ Ambitendency
- ☐ Catalepsy
- ☐ Ambivalence
- ☐ Negativism
- ☐ Posturing

Your answer is incorrect.

Explanation:

Catatonia is a syndrome of primarily psychomotor disturbances, characterised by the co-occurrence of several features of decreased, increased, or abnormal psychomotor activity. Features of decreased psychomotor activity include ambitendency (the appearance of being 'motorically stuck' in indecisive or hesitant movement) and negativism (opposing or behaving contrary to requests or instructions). Features of abnormal psychomotor activity include posturing (spontaneous and active maintenance of a posture against gravity) and catalepsy (passive induction of a posture which remains held against gravity). Ambivalence (the co-existence of two opposing impulses toward the same thing in the same person at the same time) is not a catatonic sign.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 1135.

The correct answer is: Ambivalence

Question **32**

Not answered

Marked out of 1.00

In England, the proportion of preventable suicides from a mental healthcare perspective is

Select one:

- ☐ 1 in 9
- ☐ 1 in 10
- ☐ 1 in 7
- ☐ 1 in 2
- ☐ 1 in 4

Your answer is incorrect.

About 250 suicides (1 in 4) could be prevented each year in the UK, according to the National Confidential Inquiry into suicide and homicide by people with mental illness.

The correct answer is: 1 in 4



Question **33**

Not answered

Marked out of 1.00

Which among the following psychiatric diagnosis has a higher suicidal risk?

Select one:

- ☐ Mixed anxiety and depressive disorder
- ☐ Agitated depression
- ☐ Bipolar disorder type 1
- ☐ Bipolar disorder type 2
- ☐ Unipolar major depression

Your answer is incorrect.

A high suicide risk is associated with type 2 bipolar disorder. A recent study suggests that 17% of Bipolar 1 patients, 24% of Bipolar 2 patients and 12% of Unipolar depressive patients attempt suicide. Bipolar type 2 patients does not only involve higher suicide risk but also a higher frequency of mood episode, co morbidity and rapid cycling.

The correct answer is: Bipolar disorder type 2

Question **34**

Not answered

Marked out of 1.00

Which of the following factors increases the risk of suicidal behaviour in first episode psychosis?

Select one:

- ☐ Presence of depression
- ☐ Female sex
- ☐ Belonging to a higher social class
- ☐ Poor insight
- ☐ Short duration of untreated psychosis

Your answer is incorrect.

Explanation:

Several factors are associated with a higher risk of suicidal behaviour in first-episode psychosis, including poorer neuropsychological functioning (except for working memory), poorer social cognition, more depressive symptoms, longer duration of untreated psychosis, higher awareness of the illness, poorer premorbid adjustment, and more frequent cannabis use. Depression during and after first-episode psychosis is the most significant risk factor for suicidal behaviour. 35% of first-episode patients attempt suicide, and completed suicide is most common in the early years of psychotic illness.

Ref: Sánchez-Gutiérrez T et al. Neuropsychological functioning and suicidal behaviours in patients with first-episode psychosis: A systematic review. *Acta Psychiatrica Scandinavica*. 2022 Dec;146(6):515-28.

Barnes TR et al. Evidence-based guidelines for the pharmacological treatment of schizophrenia: Updated recommendations from the British Association for Psychopharmacology. *Journal of Psychopharmacology*. 2020 Jan;34(1):3-78.

The correct answer is: Presence of depression

Question **35**

Not answered

Marked out of 1.00

Of all causes contributing to global mortality, suicide is responsible for

Select one:

- ☐ 25-30%
- ☐ 10-12%
- ☐ 1-2%
- ☐ 0.01%
- ☐ 15-20%

Your answer is incorrect.

Explanation:

For every death by suicide, approximately 20 people make suicide attempts (rates vary by country, depending on the lethality of commonly used suicide methods).

Ref: Correia R, Jackson D. Risk to self: identifying and managing risk of suicide and self-harm. BJPsych Advances. 2020;1-11.

Turecki G et al. Suicide and suicide risk. Nature reviews Disease primers. 2019 Oct 24;5(1):1-22.

The correct answer is: 1-2%

Question **36**

Not answered

Marked out of 1.00

With regard to surveys in those with suicidal ideas, the proportion of those who progress from having mere suicidal ideas to plans or attempts within one year is

Select one:

- ☐ 10%
- ☐ 20%
- ☐ 30%
- ☐ 40%
- ☐ 60%

Your answer is incorrect.

(Nock et al, The British Journal of Psychiatry (2008) 192: 98-105) According to 17 countries data as a part of WMH survey initiative, the cross-national lifetime prevalence of suicidal ideation, plans, and attempts is 9.2%, 3.1% and 2.7% . Across all countries, 60% of transitions from ideation to plan and attempt occur within the first year after ideation onset.

The correct answer is: 60%



Question **37**

Not answered

Marked out of 1.00

Which of the following is best described as a primary prevention strategy for suicide in old age?

Select one:

- ☐ Individual psychotherapy
- ☐ Development of social networks
- ☐ Crisis intervention and home treatment
- ☐ Assessment after self harm
- ☐ Treatment of a depressive episode with medications

Your answer is incorrect.

Explanation:

Social connectedness is crucial for decreasing suicide risk. Connectivity is a primary component of suicide prevention programs for older adults. Comprehensive suicide prevention initiatives promote connection to others in the community and may include health education workshops, commitments among elderly neighbours to be aware of their own and others' suicide risk, and depression screening.

Ref: Brooks SE et al. Suicide in the elderly: a multidisciplinary approach to prevention. Clinics in geriatric medicine. 2019 Feb 1;35(1):133-45.

The correct answer is: Development of social networks

Question **38**

Not answered

Marked out of 1.00

An anxious mother telephones you and asks whether antidepressants will drive her son to suicide. Which of the following is the most important question that you would ask her?

Select one:

- ☐ Has he attempted suicide in the past?
- ☐ What drug is he on?
- ☐ How many episodes of depression has her son had so far?
- ☐ Is he living with anyone at home?
- ☐ How old is he?

Your answer is incorrect.

The risk of suicidal behaviours associated with antidepressants was found to be elevated in subjects under age 25, neutral in subjects aged 25 to 64 (reduced if suicidal behaviour and ideation are considered together), and reduced in subjects aged 65 and older (so protective in elderly). Tiihonen, J et al. Antidepressants and the Risk of Suicide, Attempted Suicide, and Overall Mortality in a Nationwide Cohort. Arch Gen Psychiatry. 2006;63:1358-1367.

The correct answer is: How old is he?

Question 39

Not answered

Marked out of 1.00

The strongest known risk factor for suicide is a

Select one:

- ☐ Family history of mental health problems
- ☐ Personality disorder
- ☐ Personal history of mental health problems
- ☐ History of suicidal behaviour
- ☐ History of alcohol use disorder

Your answer is incorrect.

Explanation:

Nonfatal suicidal behaviour is among the most robust predictors of future suicidal behaviour and suicide death. Approximately 40% of people dying by suicide have previously attempted suicide.

Ref: Turecki G et al. Suicide and suicide risk. Nature reviews Disease primers. 2019 Oct 24;5(1):1-22.

The correct answer is: History of suicidal behaviour



[Refund Policy](#) | [Terms & Conditions](#)

